National Cancer Control Programme (NCCP) 00849 · clinician draft

Nivolumab, pemetrexed and cisplatin before surgery

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Day 1 · pemetrexed / cisplatin with nivolumab

Active medicines: nivolumab + pemetrexed + cisplatin

Emetogenic risk: high

Medicine	Dose / route	When
Aprepitant	125 mg orally Day 1; 80 mg orally Days 2–3	Before systemic anticancer therapy (SACT) on Day 1, then daily on Days 2–3
Ondansetron	16 mg orally	Day 1 Adjust for QT/individual risk.
Antiemetic dexamethasone reference	12 mg orally Day 1; 8 mg orally daily Days 2–4	General high-risk NCIS V8 row Reference course only: reconcile with pemetrexed rash prophylaxis and aprepitant interaction into one unambiguous prescription. Do not add both courses.
Olanzapine	5 mg orally once daily	Day 1 and Days 2–4 General high-risk NCIS reference; review sedation, falls, metabolic risk and interaction with metoclopramide. Consultant may individualise.
Metoclopramide	10 mg orally three times daily as prescribed	Subsequent days NCIS row does not specify an end day or PRN qualifier; prescriber/pharmacy must specify duration and suitability. Co-prescribing with olanzapine increases extrapyramidal risk; assess neurological contraindications and interactions.
Folic acid	350–1000 micrograms orally daily	At least five doses in seven days before first pemetrexed; throughout treatment and 21 days after the final dose
Vitamin B12 (hydroxocobalamin)	1000 micrograms intramuscular (IM)	Week before first pemetrexed and every three cycles thereafter Later doses may be on the pemetrexed day.
Pemetrexed rash prophylaxis: dexamethasone equivalent	4 mg orally twice daily	Day before, day of and day after pemetrexed This is the regimen rash schedule; prescriber/pharmacy must reconcile it with antiemetic dexamethasone into one unambiguous prescription, including aprepitant interaction. Do not simply add both courses.

Additional phase-specific support

- Take the prescribed folic acid, vitamin B12 and rash-prevention steroid plan; continue folic acid for 21 days after the final pemetrexed dose. Cisplatin needs hydration, electrolyte checks and monitoring of kidneys, hearing and nerves.
- Nivolumab can inflame healthy organs during treatment or after it stops. Report new symptoms promptly and carry your immune alert card for at least five months after the last dose.
- The prescriber specifies doses, duration, rescue treatment and interactions; follow one reconciled prescription.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00849 v2 · pNCCP 00849 pp1–7; NCCP 00317 supportive care; NCIS V8 Table 1 · NCCP NCIS antiemetics V8

Sources checked 2026-10-29 · SACTCheck v0.84.27 · Regimen-level drafts; prescriber and oncology-pharmacy verification required